

West Capacity & Patient Flow Program: Charter and Design

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Charter approved by the operating committee on January 28, 2026. Sponsor: Victor Salinas, President, Medical Center West. Program lead: Hannah Mercer, Performance Improvement. Operational lead: Aaron Foster, Capacity Management. This charter defines a six-month pilot on three medical units and the measures that will decide whether to spread it.

Background

Medical Center West ran at 72.5% average occupancy in FY2025, but monthly occupancy reached 80.2% in January 2025 and 82.7% in February 2025. On the busiest days, admitted patients waited in the ED for medical beds.

December 2025 occupancy was 75.9%. The current placement procedure and 9:30 a.m. huddle work, but most discharges happen in the afternoon, after the day's ED admissions start to queue.

Staff on 4 West, 5 West and Telemetry describe discharge planning that starts late in the stay and depends on individual physicians.

Aim

By August 31, 2026, 30% of discharges from the three pilot units will occur before 11:00 a.m., without an increase in readmission concerns raised through case management.

Secondary aim: reduce ED boarding hours attributed to the pilot units compared with the same months of 2025, recognizing that boarding also depends on season and ED volume.

Scope

In scope: 4 West Medical-Surgical (32 beds), 5 West Medical-Surgical (30 beds) and Telemetry (28 beds) at Medical Center West, from March 2, 2026.

Out of scope: the ICU, Progressive Care, 6 West, Mother-Baby, the NICU and all Hospital Fort Bend units. The placement procedure itself is not being rewritten during the pilot.

Changes to test

- Expected discharge date entered in the EHR by the end of hospital day 2 and updated daily.
- A next-day discharge list agreed on each unit by 3:00 p.m., shared with transport and pharmacy.
- Standard work for the 9:30 a.m. bed huddle: each pilot unit reports discharges planned before 11:00 a.m. and barriers to them.
- Environmental services notified when a patient leaves, rather than when the room is released in the EHR.

Measures

Measures follow the operational metric definitions standard.

- Outcome: share of discharges completed before 11:00 a.m., weekly by unit.
- Outcome: ED boarding hours attributed to pilot units, by month.
- Process: share of patients with an expected discharge date by day 2.
- Balancing: readmission concerns and complaints logged by case management.
- Context: unit midnight occupancy, including observation patients.

Team and roles

- Sponsor: President, Medical Center West.
- Program lead: Director, Performance Improvement.
- Operational lead: Manager, Capacity Management.
- Unit leads: nurse managers of 4 West, 5 West and Telemetry.
- Members: hospitalist group lead, case management manager, environmental services manager, Decision Support analyst.

Timeline

- February 2026: baseline data pull and unit training.
- March 2, 2026: pilot go-live.
- Late March 2026: next-day discharge list available in the EHR.
- June 2026: 90-day review.
- September 2026: evaluation and recommendation to the operating committee.

Risks and assumptions

Baseline figures for discharge timing will be finalized after the February data pull; the pre-pilot baseline covers only one month and will be noisy.

Summer occupancy is normally lower than winter occupancy, so comparing pilot months with February will overstate any improvement in boarding. The evaluation must account for season.

The EHR discharge list depends on the IS build queue. A paper list will be used until it is available.